Patient Care Goals

- Accurately categorize patients based on the severity of their condition to prioritize life-saving interventions and hospital transfer.
- Ensure timely identification of patients requiring immediate care or transport to prevent deterioration or death.

Inclusion & Exclusion Criteria

Inclusion Criteria

- All patients encountered by EMS personnel who require medical assessment and/or treatment, regardless of complaint or condition.

Exclusion Criteria

- Patients who have been determined as deceased on arrival (categorized as black priority).
- Patients involved in mass-casualty incidents (go to S.T.A.R.T. Triage (MCI Triage) CPG).

Key Patient Assessment Points

1. **Clinical Judgement**
   - When determining triage status, sound Clinical Judgement must be exercised. For example, an oxygen saturation of 89% may be normal for a COPD patient, it is not necessary for the patient to be triaged as Red Priority based on this parameter alone.
2. **Changes in Patient Condition and Triage Status**
   - A patient's clinical status and triage priority may change. When a patient's triage priority changes, it must be appropriately communicated and documented on the patient care report.
   - For example, a patient responding to a patient with hypoglycemia is a red priority. However, after glucose administration, becomes conscious with a normal glucose level, so the triage priority changes to yellow (or potentially green).
   - Similarly, a patient who was normo-tensive and then became hypotensive, was initially yellow priority, but then changes to red.
3. **Trauma & Mechanism of Injury (MOI)**
   - A list of significant mechanisms of injury is provided below.
   - All patients who meet significant MOI criteria must be transferred to an appropriate trauma center.
   - The MOI does not impact triage status. However, when communicating the patient's triage priority, it must be highlighted that the patient has a significant MOI. For example, "Yellow Priority with Significant MOI".
   - All patients with a significant mechanism of injury should be given a clear recommendation to be transferred to hospital.
4. **Significant Mechanism of Injury (MOI) Criteria**
   - High Risk Road Traffic Accident
     - Partial or Complete Ejection
     - Significant Cabin Intrusion:
       - >12 inches/30cm on occupant side, OR/
       - >18 inches/50cm on any side, OR/
       - Need for extrication
     - Death in same vehicle
     - Child unrestrained or in unsecured safety seat
     - Type of accident consistent with severe injury
     - Rider separated from bicycle/motorcycle/ATV/horse etc.
     - Pedestrian or Rider thrown, runover, or with significant impact
     - Speed >40km/hr (unrestrained patient) >60km/hr (restrained patient)
     - Rollover
     - Patient is a child
   - Falls
     - Fall from height >10 feet (all ages)
     - Fall >1meter (5 stairs) with head trauma

    - Low level falls in children <5 years with significant head impact
    - Low level falls in older adults >65 years with significant head impact
  - Special Populations:
    - Anticoagulant use
    - Suspicion of child abuse
    - Pregnancy >20 weeks
    - Burns in conjunction with trauma

Patient Management & Interventions

1. **Primary Survey**
   - Perform Primary Survey as per Universal Care Guideline.
2. **Determine Triage Priority**
   - Based upon findings in the Primary survey, patient triage may be able to be determined. (e.g. no breathing).
   - If the triage status is unable to be determined in the primary survey, conduct secondary survey as per Universal Care Guideline.
3. **Secondary Survey**
   - Perform secondary survey as per Universal Care Guideline.
4. **Determine Triage Priority**
   - Based upon findings in the secondary survey, determine the patient's triage category.
5. **Patient Reassessment**
   - Frequently Reassess the patient as per Universal Care CPG.
     - Patient Reassessment is required every:
       - Every 5 minutes for critical (red-priority) patients
       - Every 10 minutes for serious (yellow-priority) patients
       - Every 15 minutes for non-critical (green-priority) patients.
   - Triage Category should be updated with changes in the patients clinical condition.

Patient Safety Considerations

- Accurate triage category assignment can only be made through an effective patient assessment.
- Every patient must be fully assessed and documented in the patient care report.

Key Performance Indicators

- Assign a triage category to each patient encountered.

Documentation

- Initial Triage Category (category on arrival to patient)
- Any changes to patient triage category (if applicable)/
  - Document in additional information section if required.

Clinical Insights & Additional Guidance

- **Vital Signs Triage Guideline**
  - Vital signs in isolation are poor indicators of severity, particularly for yellow and green priority patients. A thorough patient assessment is required to adequately assess and categorize each aspect. The tables on the next page serve as a guide.

ADULT VITAL SIGNS

	Red	Yellow	Green	Yellow	Red
A V P U	P / U	V	A		
GCS	<9	10 - 14	15		
Respiratory Rate	< 10		11 – 20	21 – 29	>30
Heart Rate	< 50	51 – 60	61 – 100	101 – 149	>150
Systolic BP	< 80	80 – 100	101 - 179	180 – 200	>200
Sp02 (without 02)	< 90	91 - 94	>94		

OLDER CHILD 3 – 12 Years (95-150cm)

	Red	Yellow	Green	Yellow	Red
A V P U	P / U	V	A		
GCS	< 9	10 – 14	15		
Respiratory Rate	< 15	15 – 16	17 – 21	22 – 26	>27
Heart Rate	< 60	60 – 79	80 – 99	100 – 129	> 130
Sp02 (without 02)	< 90	91 – 94	>94		

YOUNGER CHILD < 3 Years (<95cm)

	Red	Yellow	Green	Yellow	Red
A V P U	P / U	V	A		
GCS	< 9	10 – 14	15		
Respiratory Rate	< 20	20 – 25	26 – 39	40 – 49	>50
Heart Rate	< 70	70 – 79	80 – 130	130 - 159	> 160
Sp02 (without 02)	< 90	91 – 94	>94		

RED PRIORITY PATIENT GUIDELINE

Patients with an immediately life-threatening condition - "Immediate" triage urgency

- **Neurological**
  - GCS < or equal to 9, or progressive deterioration in GCS
  - Unresponsive or responsive only to pain
  - Signs of Stroke
  - Status Epileptics
- **Airway**
  - Obstructed airway or airway needing intervention to prevent obstruction.
  - Severe Stridor
  - Airway edema
- **Breathing**
  - Severe respiratory distress or respiratory failure
  - Moderate respiratory distress refractory to initial treatment
  - SpO2 <90% on room air
- **Cardiovascular**
  - Cardiac arrest (CPR) or post-cardiac arrest (ROSC)
  - Heart Rate <50 or >150 (adult)
  - Hypotension: Systolic Blood Pressure <80mmHg (adult)
  - Hypertension: Systolic BP >200 or Diastolic BP >130 with symptoms
  - Signs of Severe Shock
  - ST elevation myocardial infarction (STEMI)
  - Chest Pain or Dysrhythmias or other ECG changes WITH significant signs of shock.
- **Obstetrics and Gynecology**
  - Pregnant with presenting fetal body part or prolapsed cord
  - Heavy PV bleeding
  - Pregnant with hypertension (Systolic BP >160, or Diastolic BP >110)
- **Other Medical**
  - Hypoglycemia <54mg/dl with symptoms
  - Hyperglycemia >324mg/dl with symptoms
  - Severe hypothermia <28°C
- **Trauma**
  - Polytrauma with signs of shock
  - Suspected fracture of 2 or more long bones
  - Penetrating Injuries to the head/neck/chest/torso or extremities proximal to the knee or elbow
  - Fractures or dislocations with signs of limb ischemia. Note: the patient may have altered sensation or movement distal to the injury but must have signs of ischemia to be Red Priority.
  - Airway burns
  - Skull deformities
  - Chest wall instability, deformity or flail segment
  - Pelvic Fracture
  - Spinal Cord Injury with motor or sensory loss
  - Crushed, degloved, mangled or pulseless extremity
  - Amputation
  - Active bleeding requiring a tourniquet or wound packing with continuous pressure
  - Burns >25% TBSA

YELLOW PRIORITY PATIENT GUIDELINE

Patients with serious but not immediately life-threatening conditions - "Urgent" triage urgency

- **Neurological**
  - Transient Ischemic Attack (TIA)
  - GCS 10-14
  - Post-seizure
- **Airway**
  - Mild or moderate Stridor
  - Breathing
  - Mild or moderate Respiratory Distress
  - Sp02 91-94% on room air
- **Cardiovascular**
  - Moderate shock:
    - Defined as: systolic BP 81–109 mmHg, cool/clammy skin, delayed capillary refill >2s, mild confusion/restlessness, tachycardia >100 but <150, AND improves with initial fluid bolus.
  - Systolic Blood Pressure 81–109 mmHg (adult).
  - 12-lead ECG abnormalities without STEMI and without severe shock
    - (e.g., NSTEMI, new LBBB, atrial fibrillation with controlled rate, other stable arrhythmias).
  - Chest pain without ST-elevation and without shock
    - (e.g., unstable angina, NSTEMI presentation with stable vitals).
- **Obstetrics & Gynecology**
  - Uncomplicated Active labor with contractions <2minutes apart
- **Trauma**
  - Polytrauma with normal or near normal vital signs
  - Isolated fracture of one long bone. This includes the shaft of the femur and compound fractures, provided there are no signs of distal limb ischemia, no shock symptoms, and external bleeding has been controlled.
  - Dislocations of joints associated without distal limb ischemia.
  - Suspected Spinal Cord Injury or spinal motion restriction required

GREEN PRIORITY PATIENT GUIDELINE

Minor Illness or Injury - "Delayed" triage urgency

- Vital signs within normal limits
- Isolated minor fractures
- Strains and Sprains
- Minor Abrasions
- 1st degree burns
- Lacerations where bleeding has been controlled (without the use of a tourniquet)
- Fever without signs of sepsis, or shock
- Headache with normal neurological findings

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